

CLINICAL INTELLIGENCE TREND REPORT

YELLOW: Closer Observation
Suggested

Patient ID: Harris Period: April 1 to April 30, 2026 Report Date: May 1, 2026 Coverage: 659/720h (92%)
Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 30 days from Apr 01 to Apr 30 ■ HR ■ Breathing



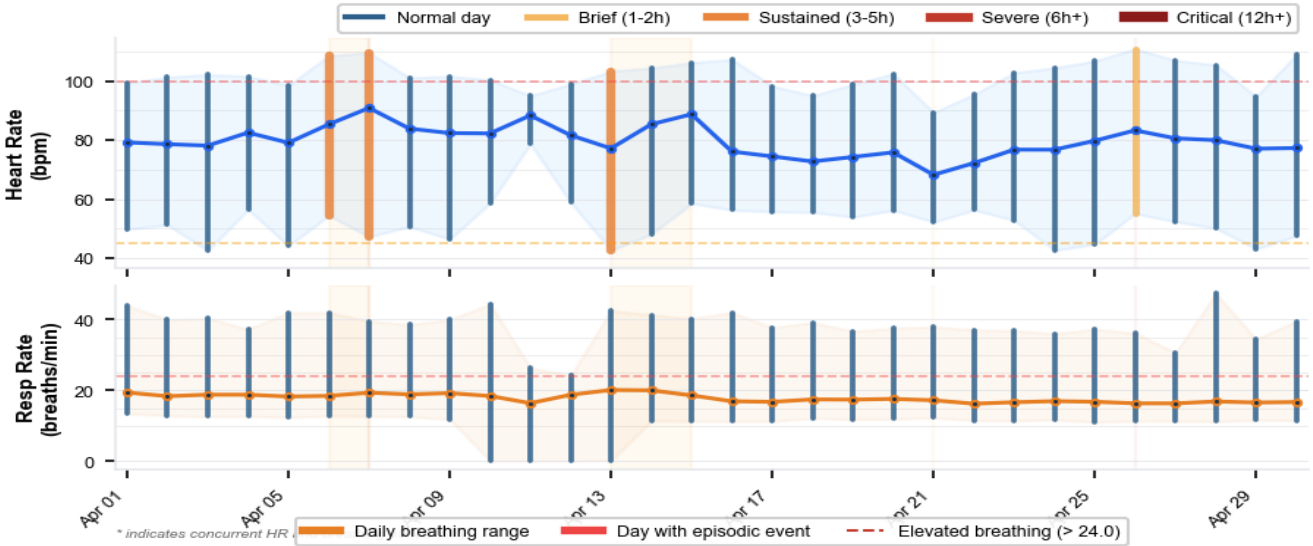
Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
12	20h	Elevated Heart Rate	<1	92%

Major Findings: Sustained elevated HR (avg 98, peak 111)

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/day	Average	Date
1	High Heart Rate	110 bpm	1h	1h	1	104 bpm	Apr 07
2	High Heart Rate	111 bpm	1h	1h	1	100 bpm	Apr 26
3	Elevated Heart Rate	109 bpm	11h	5h	2	98 bpm	Apr 06 to Apr 07
4	Elevated Heart Rate	104 bpm	6h	3h	1	97 bpm	Apr 13 to Apr 15
5	Elevated Breathing	35 brpm	1h	1h	1	24 brpm	Apr 21

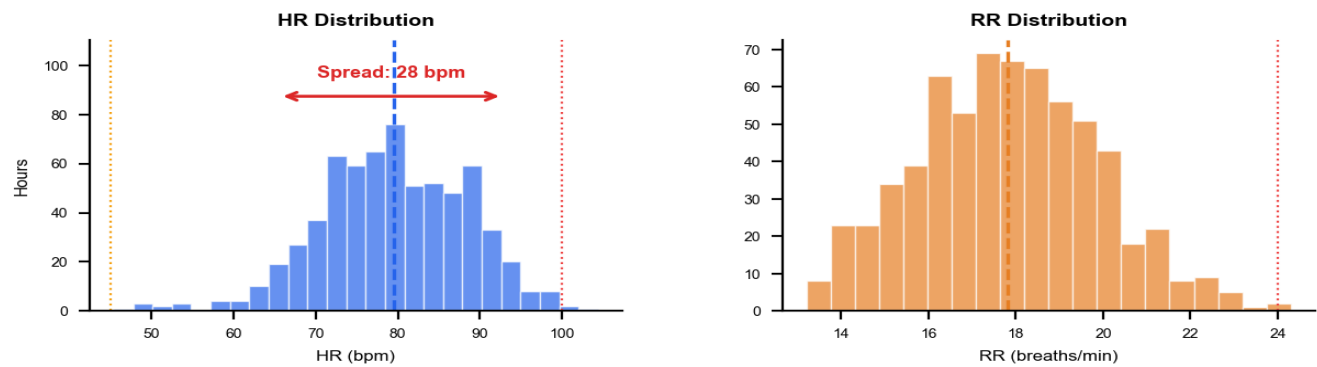
The P5 to P95 heart rate spread was 28 bpm (65 to 93), indicating significant cardiac variability.

- Clinical Pattern Observations:
- 1. **High variability:** HR spread 27 bpm (65 to 93).
 - 2. **Clustered pattern:** Episodic events on 4 of 30 days (13%).
 - 3. **Nocturnal pattern:** 5 of 6 eligible episodes during evening or night hours.

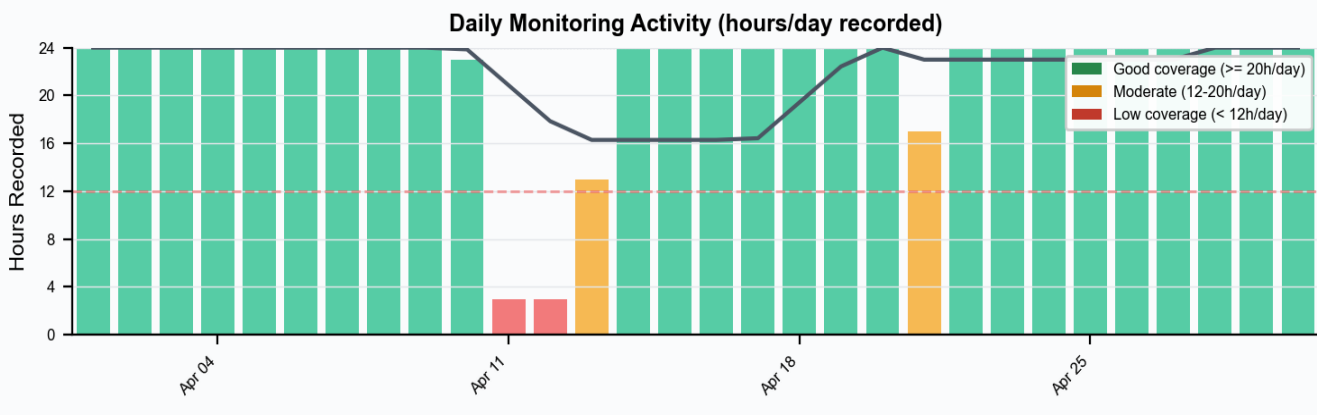


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm

Very High HR > 110 bpm
- Low HR < 45 bpm

Elevated Breathing > 24 brpm
- Elevated HR > 95 bpm

High Breathing > 30 brpm
- High HR > 100 bpm

Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.